

DPSH MUN 2026

WHO

Health for All, Leave No One Behind.

WORLD HEALTH
ORGANIZATION



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AGENDA:

“Outbreak of deadly viruses like COVID-19, EBOLA and HANTAVIRUS, from 2020-present and the failure of each country’s healthcare systems to contain and control it. With special emphasis on: Betterment of healthcare services and involvement of the UN in all major Healthcare decisions worldwide.”

Letter from the Executive Board

Greetings Delegates,

On behalf of the Secretariat and the Organising Committee, it is our immense pleasure to welcome you to the World Health Organization (WHO) Committee at DPSMUN'26.

We hope this Background Guide serves as a useful starting point for your preparation ahead of two days of engaging debate and diplomacy. Delegates are expected to have a thorough understanding of their assigned country's policies, healthcare priorities, and international stance on the agenda, while remaining informed about recent global developments.

The agenda, *"Outbreak of deadly viruses like COVID-19, Ebola and Hantavirus, from 2020–present and the failure of each country's healthcare systems to contain and control it, with special emphasis on the betterment of healthcare services and the involvement of the United Nations in major healthcare decisions worldwide"* has been chosen for its continuing global significance and its impact on international public health and governance. Please note that this Background Guide is intended only to initiate your research and should not be used as the sole source of evidence during committee sessions. We encourage you to conduct independent research and develop well-informed policy positions.

We look forward to welcoming you to DPSMUN'26. Until then, happy researching, and we wish you all the best for the committee!

Introduction

The World Health Organization (WHO), founded in 1948, serves as the United Nations' (UN) health and wellbeing coordinating body across 194 Member States. WHO focuses on matters of public health as envisioned by the UN, beginning with the adoption of the WHO Constitution at the International Health Conference in 1946. The organization seeks to address health in all aspects, recognizing health as a state of complete physical, mental, and social well-being and not merely as the absence of disease or infirmity, recognizing that the highest standard of health is a fundamental right for all people.

Governance, Mandate, Membership and Structure

WHO's constitution established the organization as a specialized agency of the UN in accordance with Article 57 of the Charter of the United Nations (1945). WHO operates within the purview of the UN Economic and Social Council (ECOSOC), coordinating global health policy and activities. The organization is made up of over 8000 health professionals across disciplinary medical fields and operates across six regional offices. WHO sets its annual agenda each year in January and convenes for week-long sessions in May. The World Health Assembly (WHA) acts as WHO's decision making body by setting the organization's policies, addressing Executive Director elections, and managing financial matters. The mandate defines WHO's role in advancing disease eradication, coordinating and directing international health programs and projects, and improving nutrition, sanitation, and other health-related conditions. WHO is also responsible for advancing medical and health-related research; promoting scientific collaboration; improving standards of training in health, medical, and related professions; as well as developing international standards for food, biological, pharmaceutical, and related products. WHO carries out projects, campaigns, and partnerships, addressing a wide range of health topics while working closely with Member States, regional organizations, and private actors. Under articles 58 and 28(I) of its constitution, WHO is tasked with carrying out appropriate emergency measures to manage epidemics and pandemics, like the COVID-19 pandemic, including taking rapid health relief measures and organizing research efforts to combat health crises. The organization is part of the UN Inter-Agency Standing Committee's (UN-IASC) Global Health Cluster, which serves as a partnership mechanism for intergovernmental, national, and nongovernmental organizations to promote health and manage public health crises through various relief, training, and program mechanisms.

WHO also ensures the response to outbreaks of highly infectious diseases, including Ebola Virus Disease (EVD), through intensified surveillance, laboratory support, vaccination programs, mounting rapid response and emergency management. As such, it collaborates with Member States, regional offices, and international partners to ensure the response to outbreaks of high consequence. The response involves deploying technical expertise, emergency medical teams, and essential medical supplies, as well as technical guidelines, laboratory networks, national health capacities, and surge capacities through training, simulation exercises, and coordination activities for effective outbreak management and control. In addition, the organization provides guidance and technical assistance for Member States' surveillance and response capacities to prevent outbreaks from occurring and stop them when they start, minimizing negative impacts on populations and economies. Furthermore, the organization is working to address the growing number of zoonotic pathogens and diseases, including Hantaviruses, considering various factors such as ecological changes, biodiversity loss, urbanization, anthropogenic encroachment, and increased trade and travel worldwide. It works closely with regional bodies, national ministries of health, and research institutes to bridge critical gaps in surveillance, laboratory capacities, data management, research, and responses while promoting the One Health approach and engaging in regional collaborative activities.

Background

The COVID-19 pandemic showed the depth of inequity in the global health system and highlighted the limitations in containing large-scale public health crises. No nation was spared, countries experiencing

ongoing armed conflict were vulnerable. The healthcare systems which were already weak, collapsed under pandemic pressures, leading to deterioration in health services, particularly maternal and child healthcare, routine vaccination programs, and emergency care.

WHO's Constitution recognizes "the highest attainable standard of health as a fundamental right of every human being," yet the pandemic demonstrated how fragile this right remains in conflict zones. The disparity was a defining feature of the pandemic.

Developed nations hoarded vaccines which left conflict-affected states with low vaccination rates. Surveillance systems in war zones were too weak to detect outbreaks early and this crisis raised questions regarding global governance and accountability.

The challenges encountered during the COVID-19 pandemic also resonate with previous experiences in managing Ebola Virus Disease (EVD) outbreaks. Lessons from treating EVD patients reveal that the virus remains a significant threat in several countries, contributing to persistent health security issues in vulnerable populations. Although considerable progress has been made in developing treatment methods and managing outbreaks, such as the recent cases in the Democratic Republic of Congo, the experiences of EVD outbreaks still serve as a critical lesson for combating new outbreaks like COVID-19. In particular, the occurrence of multiple Ebola outbreaks indicates the insufficiency of disease surveillance systems, laboratory capabilities, and general health infrastructure in conflict-affected regions, necessitating international collaboration to prevent and control future outbreaks. Moreover, the recent Hantavirus outbreak added to the list of concerning zoonotic diseases, drawing attention to the potential impacts of ecological damage, environmental change, and increased human activity on the spread of pathogens. Despite primarily affecting local populations, outbreaks of new viruses emphasize the importance of timely detection, comprehensive disease surveillance, and international collaboration in managing and preventing emerging health threats. Ultimately, the challenges posed by COVID-19, Ebola, and Hantavirus illustrate the need for improved healthcare systems, increased global health security, and better international relations to ensure an adequate response to future health crises. Therefore, it is evident that the challenges of pandemics cannot be addressed by one country alone, requiring the collaboration of various nations, international organizations, and entities such as the World Health Organization and the United Nations

Equity to Acquire Vaccines and Discrimination in Distribution

The COVID-19 pandemic highlighted global disparities in vaccine distribution, contradicting the WHO's mandate to promote health equity. Of the 832 million COVID-19 vaccines administered early in the pandemic, 82% were distributed to high-income Member States. The already limited health services were disrupted when the pandemic hit, which caused a drop in global vaccine coverage for children under 12 by over 5%. An estimated 25 million children under the age of one missed important early childhood vaccines, the highest since 2009. There was a particular disparity where girls faced discrimination, with rates for important childhood vaccinations, like the HPV vaccine, dropping by over 10% from 2019 to 2020.

WHO has addressed this challenge with the COVID-19 Vaccine Global Access (COVAX) initiative to promote equitable distribution of vaccines across the globe. The Global Immunization Agenda 2030 prioritizes over 20 preventable diseases and focuses distribution mainly in infants across the developing world.

Inability to Stop the Spread and Detect the Threat

The International Health Regulations (IHR) had critical flaws which were exposed during the initial phase of the pandemic. The IHR relied on individual states to quickly and transparently report a potential public health emergency of international concern (PHEIC), this clearly failed which led to a delayed recognition of the COVID-19 pandemic. WHO used the Early Warning, Alert, and Response System (EWARS). EWARS is a system which is designed to detect and rapidly respond to public health threats, particularly disease outbreaks, in emergency or vulnerable settings.

Health Deterioration in Conflict Zones

The effects of COVID-19 were magnified in states experiencing constant war or political fragility, pushing already strained health services toward total collapse. Countries which were in conflict such as Syria, Yemen, Afghanistan couldn't shift resources in response to the pandemic as they were dealing with war displacement. The already fragile medical supply chain shattered due to the border closing and global competition for PPE and ventilators. There was a large increase in preventable deaths; services such as maternal care dropped across 38% of member states, increasing stillbirth and maternal mortality rates in developing nations. There are millions of people who live in conflict zones as refugees in overcrowded camps where infectious diseases can spread rapidly. Due to the already collapsed immunization and sanitation programs in such areas, the people residing are at risk of various other diseases like measles, cholera and polio.

Discrimination for Vaccination and Priority

Discrimination in vaccination during the COVID-19 pandemic was a critical failure of the global health order.

The most significant form of discrimination occurred at the international level. High-income countries secured the majority of the early vaccine supply through bilateral deals (a formal agreement between two parties that creates mutual promises and obligations) with pharmaceutical companies. This resulted in a staggering global disparity, where 82% of vaccines were administered in high income countries early on and vaccination rates were approximately 1 in 4 in richer nations versus 1 in 500 in poorer nations.

This discrimination in vaccination was not only a moral issue but also a significant health issue as large unvaccinated populations in developing nations acted as pool for viral mutation, threatening the efficacy

of vaccines globally. To combat this, The COVAX initiative was launched by WHO, though it struggled to compete with the purchasing power of wealthy nations.

Within countries, the preexisting social inequalities were further exacerbated when it came to vaccination. In many countries, racial and ethnic minority groups faced higher rates of infection, hospitalization, and death. Groups like refugees, internally displaced persons, and those in slums were consistently pushed to the back of the queue, making them highly vulnerable to outbreaks.

Recurring Ebola Outbreaks

Since its discovery in the Ebola River in what is now the Democratic Republic of the Congo (DRC) in 1976, Ebola Virus Disease (EVD) has been among the world's deadliest viral haemorrhagic fevers. With a typical case fatality rate (CFR) of approximately 50%, EVD outbreaks have seen CFRs range from 25% to as high as 90% in certain instances. Furthermore, the DRC has had the most significant number of recorded Ebola outbreaks, with seventeen outbreaks occurring since the virus's discovery. It appears that Ebola has been a persistent threat to the Republic's population rather than a one-off incident.

It is evident that recurrent outbreaks have revealed critical healthcare system vulnerabilities, particularly in the Republic, where many of the outbreaks have occurred. These challenges include a lack of disease surveillance infrastructure, the capacity, and trained personnel in managing such cases, limited laboratory capacity, and insufficient infection prevention and control measures. In addition, several of these Ebola outbreaks occurred simultaneously with conflict and disaster events, with significant numbers of displaced persons and extensive socio-economic disruptions, making tracing and surveillance challenging. Moreover, each outbreak had distinct strains of the virus, which adds to the challenge. While there are now two authorized vaccines, Ervebo and Mvabea/Zabdeno, for the Zaire ebolavirus, there is no approved therapy for the Bundibugyo ebolavirus, and treatment is primarily supportive during an outbreak.

The 2026 Bundibugyo Ebola Outbreak

The 2026 Ebola outbreak caused by Bundibugyo ebolavirus (BDBV) is one of the largest Ebola outbreaks since the 2014-2016 West African epidemic. The World Health Organization (WHO) was alerted on 5 May 2026 to a possible outbreak of an unknown disease in Ituri province of the Democratic Republic of the Congo (DRC). The Institut National de Recherche Biomédicale (INRB) positively identified Bundibugyo virus on 15 May 2026. An emergency meeting of the IHR Emergency Committee was held on 17 May 2026 following the declaration of a Public Health Emergency of International Concern (PHEIC) by the WHO Director-General under Article 12 of the IHR (2005).

The committee acknowledged that while the outbreak met the threshold for a PHEIC, it did not qualify for a Pandemic Emergency under the amended Pandemic Agreement of 2024, representing one of the first significant applications of the new Pandemic Framework. Unlike Zaire ebolavirus, which has licensed vaccines such as Ervebo, there are currently no approved vaccines or therapeutics for BDBV. The healthcare workers provided supportive care while the WHO, research institutes, and pharmaceutical companies worked on clinical trials for a vaccine and potential treatments. Furthermore, the outbreak highlighted critical surveillance gaps, with the WHO reporting a delay of approximately four weeks between the first suspected case and laboratory confirmation and challenges differentiating between similar

illnesses like malaria, typhoid fever, and other endemic conditions. The conflict, humanitarian crisis, displaced populations, and increased regional trade presented additional challenges to case detection, contact tracing, and healthcare access in the affected region. The WHO AFRO regional office initiated an epidemiological and laboratory investigation, emergency medical response, cross-border surveillance, and risk communication in coordination with neighboring states, humanitarian partners, and local authorities. The temporary recommendations adopted by the IHR (2005) Emergency Committee included advising Member States to step up surveillance at points of entry, rapidly identify sick travelers, and intensify laboratory testing and reporting while avoiding unnecessary interference with international traffic and trade.

As of late-July 2026, the outbreak has become the largest-ever recorded Bundibugyo virus outbreak with more than 2,500 confirmed cases and over 1,000 confirmed deaths in the DRC and 20 confirmed cases in Uganda successfully managed with strict public health measures. Moreover, outside Africa, the outbreak resulted in several imported cases in France, indicating the potential for distant regions to experience sporadic cases or localized outbreaks despite the disease's limited transmissibility. The 2026 Ebola outbreak poses a challenging test for the WHO and serves as a sobering reminder of the vulnerabilities in the international health security system in responding to high-consequence diseases with high fatality rates.

Healthcare System Challenge

Ebola outbreaks have always revealed the weaknesses of the healthcare systems in affected countries. Firstly, the absence of Universal Health Coverage (UHC) often leaves the population without access to healthcare facilities under strain. The World Health Organization (WHO) reports that inadequate primary healthcare leads to delayed diagnoses, poor infection prevention and control, and an inability to isolate cases, resulting in further transmission before control measures can be effectively implemented. During the 2014-2016 outbreak, over 500 healthcare workers lost their lives to Ebola, severely impacting the response capacity of local health systems. Additionally, routine healthcare services such as childhood immunizations, maternal care, malaria treatment, HIV care, and TB treatment were significantly disrupted, leading to thousands of preventable deaths from other causes.

A lack of diagnostic infrastructure, such as laboratories, is another significant challenge. The majority of countries are reliant on reference laboratories, leading to delays in diagnosis and treatment, as well as impeding contact tracing efforts. The WHO has called for decentralized testing and genomic surveillance to improve early detection and response capabilities. In addition, there is a critical shortage of intensive care, isolation, ambulances, medical oxygen, and essential medicines.

Finally, communities often suffer from fear, stigma, and mistrust of authorities, resulting in the concealment of cases and the failure to follow recommended safety measures. This can lead to the continued spread of the virus, as seen in various instances where individuals did not adhere to safe burial practices or seek treatment.

Threat of Hantavirus

Hantavirus has become a major public health threat, with the problem being exacerbated by the fact that the virus is zoonotic, which means that the infection is transmitted from animals to humans. Unlike Ebola,

and COVID-19, which are largely contracted from person to person, Hantaviruses are mainly transmitted through the air from rodents to humans. However, there are exceptions, one of which is Andes virus, also known as the Hantavirus that affects South America. This particular virus can be transmitted from person to person, albeit very rarely, which makes it a serious threat to the general population. According to the Congressional Research Service (CRS), the data on the spread of the virus show that the number of reported cases is increasing. For instance, there were 20 cases in the US in 2024, 40 cases in 2025, and several more cases in the first months of 2026. Additionally, the Pan American Health Organization (PAHO) reported that “Argentina, Chile, and Paraguay each recorded over 100 cases of hantavirus between June 2025 and May 2026 - almost double the number than in the same period last year.” Researchers believe that climate changes, including warm temperatures, allow the virus to affect a greater number of individuals. Moreover, the fact that more people are living in rural areas has also contributed to the increased number of reported cases. The issue is made worse by the fact that over 90% of infections occur in agricultural settings, which shows that the problem is largely neglected outside of hospital settings.

The 2026 MV Hondius Hantavirus Outbreak

The 2026 MV Hondius hantavirus outbreak was one of the largest travel-related outbreaks of zoonoses and an example of how local health issues can become international emergencies due to modern transportation opportunities. The Dutch-registered expedition cruise ship called Hondius left Ushuaia, Argentina, on April 1, 2026, carrying 147 passengers and 25 crew members of 23 different nationalities. The vessel arrived at South Georgia and South Sandwich Islands, Antarctica, Tristan da Cunha, Saint Helena, Ascension, and returned to Ushuaia in early May. The World Health Organization (WHO) was alerted on May 2nd, 2026, when the ship reached Tenerife, Canary Islands, after two passengers died, and another was hospitalized in severe condition. The infection was determined to be the Andes virus (ANDV), one of the hantaviruses that may be transmitted from person to person, on May 6th.

The response to the outbreak was complicated by the status of the cruise ship as a sovereign jurisdiction and the fact that no country had jurisdiction over the passengers of different nationalities. The response was coordinated by the World Health Organization, European Centre for Disease Prevention and Health Information, and the U.S. CDC in collaboration with national health authorities. The affected passengers were evacuated and repatriated via charter flights after the ship arrived in Tenerife, and those exposed to the virus underwent quarantine, while American citizens were sent to the Nebraska biocontainment unit. The response was challenging since it involved passengers of different nationalities, but the number of infected individuals was relatively small for such a large-scale initiative.

The international response to the 2026 MV Hondius outbreak was determined by the International Health Regulations (2005). In particular, member states are obliged to notify WHO of PHEIC, which may necessitate the adoption of emergency measures, and to provide it with appropriate information. Additionally, they should notify all states potentially affected by the event and collaborate on coordinating measures. In the context of the outbreak on the Hondius, the WHO issued Disease Outbreak News statements and conducted contact tracing. Moreover, the organization provided recommendations to states in collaboration with the CDC and ECDC, and ensured surveillance without restricting trade and travel between countries. The outbreak was officially over on July 20th, 2026, after 13 cases of infection and three fatalities. The quarantine was lifted without the appearance of new cases.

Failure of Healthcare Systems to Prevent these issues

The COVID-19, Ebola, and Hantavirus outbreaks highlighted the shortcomings of healthcare systems in both developed and developing countries. Despite the high level of medical technologies and healthcare infrastructure in modern society, the ability of states to respond to health security challenges has proven to be inadequate. For example:

- Italy turned out to be the first epicenter of the coronavirus outbreak in Europe. There was an outbreak of cases in the Lombardy region. Hospitals faced a shortage of ICU capacity and had to allocate ICU facilities for those who were in critical condition owing to ventilators' and ICU beds' deficiency. Many healthcare professionals got infected because of the shortage of protective equipment and their long-term exposure to the disease, despite Italy having one of Europe's advanced healthcare systems.
- The total number of deaths in America due to COVID-19 stood at about 1.2 million which was the highest number of national deaths anywhere in the world. Shortages of testing kits, poor contact tracing methods, and lack of coherent policies hampered the containment process.
- In the wave of the Delta variant, India's healthcare witnessed an unparalleled situation where hospitals lacked oxygen, ICU beds, medicines, and even healthcare professionals. In major cities like Delhi and Mumbai, there have been cases of patients queuing up outside the hospitals since the bed availability was not sufficient. Lack of oxygen resulted in many preventable deaths due to the same reason in India, despite India's later success in large-scale vaccine production.
- The Ebola outbreak in West Africa highlighted serious shortcomings in the health system in the affected countries – Guinea, Liberia, and Sierra Leone. More than 500 healthcare workers succumbed to Ebola after caring for the infected individuals, further straining an already fragile health infrastructure. Health facilities stopped delivering routine healthcare services such as vaccinations, care for mothers and their babies, malaria treatment, HIV care, and TB management.
- In Argentina, the Andes virus caused an outbreak in Epuyén. This region has seen the first cases of the rare human-to-human transmission of the disease. The symptoms of the virus were similar to the flu, resulting in many cases going undiagnosed before entering isolation. The authorities performed rigorous quarantine procedures and contact tracing to ensure that the virus was contained. Although the healthcare system in Argentina is well-developed, the outbreak revealed vulnerabilities in the country's response to rural outbreaks and lack of resources to combat such diseases.

Conclusion

The COVID-19 pandemic served as a profound demonstration that the existing global health architecture is no longer fit for purpose. The IHR's reliance on state goodwill and self-reporting proved insufficient, creating critical delays in detection and allowing political self-interest to supersede global public health. Simultaneously, the vaccine and discrimination denied the fundamental right to health for the most vulnerable populations, particularly women and children, allowing the virus to mutate and prolong the crisis for everyone. Moving forward, the WHO and its Member States must pivot from this reactive

framework and strengthen its legal authority, institutionalize equity making it accessible to all nations, regardless of economic status and protect the vulnerable. Ebola epidemics and Hantavirus infections are additional examples that indicate that global health security cannot depend only on the emergency response to the existing epidemics. Continuous strengthening of national health care systems through funding of disease surveillance, laboratory capacity, health workforces, research, and emergency preparedness should be prioritized in the long run. Cooperation between the Member States, WHO, and the UN becomes crucial in terms of providing timely exchange of information, equal access to vaccination, testing, treatment, and international response. Although there are some reforms such as 2024 amendments of IHR and WHO Pandemic Agreement that can become a good chance to enhance global health security, their success will ultimately depend on political commitment and financing.

Questions to Consider

- Q. How can the WHO utilize public-private partnerships to better achieve equitable vaccine distribution and health service goals in developing countries?
- Q. What specific preventive actions and mechanisms, beyond the existing IHR, should be adopted to manage possible or onset public health crises before they expand globally?
- Q. How should the WHO and its partners address the unique logistical and security challenges of delivering lifesaving vaccines and chronic care in countries under constant war or active conflict?
- Q. What reforms are necessary to eliminate disparities in access to healthcare services and vaccinations, ensuring a "One World, One Health" approach?
- Q. Beyond immediate medical aid, what role must the WHO play in rebuilding the mental health infrastructure of countries devastated by prolonged crisis and pandemic stress?

Helpful Links

- 1) <https://www.who.int/>
- 2) <https://www.unwomen.org/en/digital-library/publications/2021/02/report>
- 3) <https://www.who.int/initiatives/act-accelerator/covax>
- 4) <https://www.unicef.org/coronavirus/covid-19>
- 5) <https://www.who.int/emergencies/disasters-and-health-emergencies>
- 6) <https://www.unwomen.org/en/digital-library/publications/2021/02/report-on-the-un-women-global-response-to-covid-19>
- 7) <https://www.who.int/news-room/fact-sheets/detail/ebola-disease>
- 8) <https://www.who.int/initiatives/genomic-surveillance-strategy>
- 9) <https://www.reuters.com/business/healthcare-pharmaceuticals/congo-says-more-than-1000-die-ebola-confirmed-cases-rise-2536-2026-07-23/>
- 10) <https://www.who.int/news/item/17-05-2026-epidemic-of-ebola-disease-in-the-democratic-republic-of-the-congo-and-uganda-determined-a-public-health-emergency-of-international-concern>

- 11) <https://www.who.int/emergencies/disease-outbreak-news/item/2026-DON611>
- 12) <https://www.who.int/news/item/22-05-2026-first-meeting-of-the-ihf-emergency-committee-regarding-the-epidemic-of-ebola-bundibugyo-virus-disease-in-the-democratic-republic-of-the-congo-and-uganda-2026-temporary-recommendations?>
- 13) <https://www.who.int/emergencies/situations/ebola-outbreak---drc-2026?>
- 14) <https://www.afro.who.int/countries/uganda/news/uganda-begins-countdown-end-ebola-outbreak>

